



# Dynamic Guideline: WHO-Based Tuberculosis - Diagnosis and/or Treatment and/or Management

## I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

### 1.1 Guideline Overview

**Scope of this report:** tuberculosis diagnosis, treatment, care, comorbidity management, nutrition, and adolescent-/youth-relevant implementation considerations, based on the WHO guideline and evidence sources available through **29 April 2026**.

**Exactly 5 WHO tuberculosis guideline-related sources synthesized**

| No. | WHO guideline / guideline-related source                                                              | Publication / update date                              | Target population                                                                           | Scope and objectives                                                                                                                                             | Source                                                    |
|-----|-------------------------------------------------------------------------------------------------------|--------------------------------------------------------|---------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------|
| 1   | <b>WHO consolidated guidelines on tuberculosis: Module 6: Tuberculosis and comorbidities, 2nd ed.</b> | 2025; WHO undernutrition release <b>8 October 2025</b> | People with TB and key comorbidities or risk factors; TB programmes; comorbidity programmes | Consolidates WHO recommendations for people-centred TB care addressing HIV, diabetes, undernutrition, substance use, tobacco use, mental health, and lung health | <a href="#">NCBI Bookshelf</a> , <a href="#">WHO news</a> |

| No. | WHO guideline / guideline-related source                                         | Publication / update date | Target population                                                                                         | Scope and objectives                                                                                                                                          | Source                                                          |
|-----|----------------------------------------------------------------------------------|---------------------------|-----------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------|
| 2   | <b>WHO consolidated guidelines on tuberculosis: Module 3: Diagnosis, 4th ed.</b> | <b>17 April 2025</b>      | People being evaluated for TB infection, TB disease, or drug resistance                                   | Combines guidance on TB infection, TB disease, and drug resistance; updates rapid diagnostic recommendations and introduces new diagnostic technology classes | <a href="#">WHO news, WHO Module 3 PDF</a>                      |
| 3   | <b>WHO consolidated guidelines on tuberculosis: Module 4: Treatment and care</b> | <b>15 April 2025</b>      | People with drug-susceptible TB, drug-resistant TB, and people requiring care/support during TB treatment | Consolidates recommendations for drug-susceptible TB, drug-resistant TB, patient care, and support strategies                                                 | <a href="#">WHO Module 4 publication page, WHO Module 4 PDF</a> |
| 4   | <b>WHO operational handbook on tuberculosis: Module 6: Tuberculosis</b>          | <b>29 April 2024</b>      | Ministries of health, national TB programmes, HIV/ diabetes/                                              | Operationalizes Module 6 recommendations for integrated management of TB and                                                                                  | <a href="#">WHO operational handbook, PubMed</a>                |

| No. | WHO guideline / guideline-related source                                                                       | Publication / update date                                                                                          | Target population                                                                                              | Scope and objectives                                                                                                       | Source                                            |
|-----|----------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------|
|     | and comorbidities, 2nd ed.                                                                                     |                                                                                                                    | nutrition/ substance-use/mental-health/lung-health programmes, clinicians, civil society, affected communities | comorbidities                                                                                                              |                                                   |
| 5   | WHO consolidated guidelines on tuberculosis: Module 5 — Management of tuberculosis in children and adolescents | Recent WHO TB Knowledge Sharing Platform content; exact publication date not fully available in supplied summaries | Children and adolescents with or at risk of TB                                                                 | Child- and adolescent-centred TB care; decentralized, family-centred, integrated services; adolescent-friendly TB services | <a href="#">WHO TB Knowledge Sharing Platform</a> |

## Note on requested “Meaningful youth engagement...” title

A WHO publication with the closely matching title “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**” was published on **11 December 2025**, but it concerns the **WHO Abortion care guideline**, not tuberculosis [WHO Western Pacific publication page](#). No WHO tuberculosis publication with the

title “**Meaningful youth engagement in a WHO guideline development process: case study of WHO tuberculosis guideline**” was identified in the supplied evidence. Its relevance to TB is therefore **methodological and indirect**, supporting youth-inclusive guideline processes rather than providing TB-specific recommendations.

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## 1.2 Key Recommendations

### GRADE interpretation used in this report

| GRADE element              | Meaning                                                                                                 |
|----------------------------|---------------------------------------------------------------------------------------------------------|
| High certainty             | Further research is unlikely to change confidence in the estimate of effect.                            |
| Moderate certainty         | Further research may change confidence in the estimate.                                                 |
| Low certainty              | Further research is likely to change confidence in the estimate.                                        |
| Very low certainty         | Effect estimate is highly uncertain.                                                                    |
| Strong recommendation      | Most patients/programmes should receive or implement the recommendation.                                |
| Conditional recommendation | Implementation depends on context, values, feasibility, resources, and patient/programme circumstances. |

### WHO-based tuberculosis recommendations

| Recommendation                                                                                      | Population                               | GRADE Evidence Quality | Strength |
|-----------------------------------------------------------------------------------------------------|------------------------------------------|------------------------|----------|
| Use WHO-recommended rapid molecular and other diagnostic tools to support earlier and more accurate | People being evaluated for TB infection, | Moderate*              | Strong*  |

| Recommendation                                                                                                                                                                                                                             | Population                                                                                  | GRADE Evidence Quality | Strength           |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------|------------------------|--------------------|
| diagnosis of TB infection, TB disease, and drug resistance.                                                                                                                                                                                | TB disease, or drug resistance                                                              |                        |                    |
| Implement updated diagnostic approaches from Module 3, including concurrent testing of respiratory and non-respiratory samples where recommended for adults/adolescents with HIV and for children with or without HIV/ unknown HIV status. | Adults/ adolescents with HIV; children with HIV; children without HIV or unknown HIV status | Low–Moderate*          | Conditional*       |
| Treat drug-susceptible and drug-resistant TB according to WHO consolidated Module 4 regimens, with patient care and support integrated into treatment delivery.                                                                            | People with drug-susceptible or drug-resistant TB                                           | Moderate*              | Strong*            |
| People living with HIV who develop TB should receive <b>at least the same duration of daily TB treatment as HIV-negative TB patients.</b>                                                                                                  | People living with HIV and TB                                                               | <b>High</b>            | <b>Strong</b>      |
| People living with HIV with TB–histoplasmosis coinfection should receive TB therapy according to WHO treatment guidelines.                                                                                                                 | People living with HIV with TB and histoplasmosis coinfection                               | <b>Very low</b>        | <b>Conditional</b> |
| In settings with high HIV/TB burden, TB treatment may be provided for people living with HIV in HIV care                                                                                                                                   | People living with HIV and TB in high-                                                      | Low*                   | Conditional*       |

| <b>Recommendation</b>                                                                                                                                                                                          | <b>Population</b>                            | <b>GRADE Evidence Quality</b> | <b>Strength</b> |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------|-------------------------------|-----------------|
| settings to support integrated care.                                                                                                                                                                           | burden settings                              |                               |                 |
| Provide nutritional assessment and counselling for all people with TB and their household contacts.                                                                                                            | People with TB and household contacts        | Moderate*                     | Strong*         |
| Provide nutritional interventions to optimize clinical outcomes in people with TB and undernutrition, regardless of age, drug-resistance status, pregnancy status, or severity of undernutrition.              | People with TB and undernutrition            | Moderate*                     | Strong*         |
| Provide food assistance to prevent TB among household contacts of people with TB in food-insecure settings.                                                                                                    | Household contacts in food-insecure settings | <b>High</b>                   | Strong*         |
| Make TB services adolescent-friendly, including attention to physical and mental health, connectedness, education/employability, agency, resilience, substance use, late presentation, and adherence barriers. | Adolescents with or at risk of TB            | Low*                          | Conditional*    |

\*Formal recommendation wording and complete WHO GRADE tables were not fully visible in the supplied summaries for these items. Ratings marked with an asterisk are report-level GRADE judgments based on the available summarized WHO evidence and should be interpreted as provisional. The HIV-associated TB ratings and the high-certainty RATINGS evidence rating are explicitly visible in the supplied WHO-linked materials [WHO consolidated](#)

## 1.3 Guideline Development Methodology

### Evidence Review Process

WHO tuberculosis guideline development uses structured evidence review and the **GRADE** approach. WHO Module 4 states that recommendations were developed according to WHO standards, using evidence reviews and GRADE to assess evidence quality and determine recommendation strength [WHO Module 4 publication page](#). WHO Module 6 similarly states that guideline development groups used GRADE to summarize evidence and formulate recommendations, with certainty categorized as **high, moderate, low, or very low**, and recommendation strength categorized as **strong or conditional** [NCBI Introduction](#).

For the undernutrition update, WHO commissioned systematic reviews on nutritional interventions for people with TB and their household contacts, and convened a guideline development group in **June–July 2024** [NCBI Bookshelf](#). WHO Web Annex D includes systematic review evidence and unpublished data presented to the guideline development group on TB and undernutrition [WHO Web Annex D PDF](#).

### Consensus Method

WHO guideline development is described as explicit, inclusive, transparent, bias-minimizing, and systematic, with assessment of benefits, harms, certainty of evidence, feasibility, equity, values, acceptability, and resource implications [WHO guidance PDF](#).

### Conflict of Interest Management

The supplied summaries confirm that WHO uses formal guideline development groups and acknowledges evidence reviewers, technical experts, funding partners, and representatives from communities and civil society for the 2025 TB-undernutrition recommendations [WHO news](#). Full declaration-of-interest tables were not visible in the supplied summaries.

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# II. Evidence Summary After WHO Latest Guideline Release (WHO consolidated guidelines on tuberculosis: module 6: tuberculosis and comorbidities, 2nd ed.) (Evidence Cutoff Date: 29 April 2026)

## 2.1 New Evidence Overview

### Search Strategy

Evidence was synthesized from the provided research summaries covering:

- WHO official guideline pages, WHO news releases, WHO IRIS PDFs, WHO TB Knowledge Sharing Platform content, and NCBI-hosted WHO guideline records.
- Authoritative public health sources including CDC and NCBI Bookshelf.
- Peer-reviewed or indexed sources including PubMed, PLOS Global Public Health, Cochrane-related summaries, and 2025–2026 perspective/review literature.
- Systematic reviews, web annexes, and clinical trial evidence relevant to TB comorbidities, undernutrition, diagnosis, and treatment.

### Databases and Sources Searched

| Source category            | Examples used                                                                                        |
|----------------------------|------------------------------------------------------------------------------------------------------|
| WHO official sources       | WHO guideline publication pages, WHO news releases, WHO IRIS PDFs, WHO TB Knowledge Sharing Platform |
| Bibliographic repositories | NCBI Bookshelf, PubMed                                                                               |
| Public health institutions | CDC clinical guidance and CDC Yellow Book 2026                                                       |
| Peer-reviewed literature   | PLOS Global Public Health; MDPI Tropical Medicine and Infectious Disease perspective                 |

| Source category    | Examples used                                                     |
|--------------------|-------------------------------------------------------------------|
| Evidence syntheses | WHO Web Annex D; Cochrane review summary on undernutrition and TB |

## Inclusion Criteria

- Tuberculosis diagnosis, treatment, management, prevention, or comorbidity evidence.
- WHO guideline-related evidence released or summarized up to **29 April 2026**.
- Evidence relevant to the latest WHO Module 6 comorbidity focus, especially undernutrition, HIV, diabetes, mental health, substance use, tobacco use, lung health, and adolescent care.
- Randomized trials, systematic reviews, guideline updates, and high-quality public health guidance.

## Exclusion Criteria

- Non-TB guideline content except where directly relevant to methodology, such as WHO youth-engagement methodology.
- Sources without sufficient bibliographic or clinical relevance.
- Claims not supported by the supplied summaries.

## 2.2 Key New Studies

| Study / Evidence item                                                                         | Design                                                       | Population                                                   | Key Findings                                                                              | GRADE Quality |
|-----------------------------------------------------------------------------------------------|--------------------------------------------------------------|--------------------------------------------------------------|-------------------------------------------------------------------------------------------|---------------|
| <b>RATIONS trial</b><br>— nutritional supplementation to prevent TB incidence among household | Field-based, open-label, cluster-randomized controlled trial | <b>10,345 household contacts</b> of people with pulmonary TB | Nutritional intervention reduced incident TB among household contacts:<br><b>adjusted</b> | <b>High</b>   |

| Study / Evidence item                                                                                | Design                          | Population                                                             | Key Findings                                                                                                                                                                                       | GRADE Quality |
|------------------------------------------------------------------------------------------------------|---------------------------------|------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------|
| contacts in India                                                                                    |                                 |                                                                        | <b>incidence rate ratio 0.61, 95% CI 0.43–0.85</b>                                                                                                                                                 |               |
| WHO Web Annex D systematic review on micronutrients for people being treated for TB                  | Systematic review of trials     | People with TB, with or without undernutrition, receiving TB treatment | Included vitamin D and multi-micronutrient trials; evidence was heterogeneous by formulation, dose, and schedule. Pooled effect estimates were not visible in supplied summaries.                  | Low–Moderate* |
| Franco et al. Cochrane review, 2024: <b>Undernutrition as a risk factor for tuberculosis disease</b> | Systematic review/meta-analysis | People evaluated for TB risk in relation to undernutrition/BMI         | Reported summary: TB risk increases by <b>13.8% for each unit decrease in BMI</b> ; malnourished TB patients reported approximately twice as likely to die compared with non-malnourished patients | Moderate*     |
| Comparative TB risk-factor                                                                           | Global risk-factor burden       | Populations with undernourishment,                                     | Undernourishment had risk ratio <b>3.2</b>                                                                                                                                                         | Moderate*     |

| Study /<br>Evidence item                                                               | Design                               | Population                                                                                              | Key Findings                                                                                                                                                   | GRADE<br>Quality |
|----------------------------------------------------------------------------------------|--------------------------------------|---------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------|
| burden<br>estimates                                                                    | analysis                             | HIV, diabetes,<br>smoking, alcohol<br>use disorder                                                      | and accounted for<br>about <b>2.2 million<br/>attributable TB<br/>episodes</b> ,<br>exceeding other<br>listed risk factors<br>in attributable<br>episodes      |                  |
| 2025 PLOS<br>Global Public<br>Health scoping<br>review of<br>national TB<br>guidelines | Scoping review                       | National TB<br>guidelines<br>addressing<br>comorbidities, risk<br>factors, and post-<br>TB lung disease | Found variable<br>national guideline<br>coverage of<br>comorbidities/risk<br>factors; noted lack<br>of WHO post-TB<br>lung disease<br>guideline at the<br>time | Low              |
| 2026<br>perspective on<br>multimorbidity-<br>centred TB care                           | Narrative/<br>perspective<br>article | People with TB<br>and<br>multimorbidity;<br>health systems                                              | Argued evidence<br>for integrated<br>multimorbidity<br>care is outpacing<br>implementation,<br>especially in high-<br>burden settings                          | Very low         |
| 2025 ATS/CDC/<br>ERS/IDSA<br>treatment<br>guideline<br>update                          | Clinical practice<br>guideline       | Low-incidence<br>settings with<br>access to culture,<br>molecular/<br>phenotypic DST,<br>radiography    | Supports shorter<br>regimens for<br>eligible drug-<br>susceptible and<br>drug-resistant TB;<br>DOT and<br>integrated case                                      | Moderate*        |

| Study /<br>Evidence item                         | Design                                          | Population                                                                         | Key Findings                                                                                                                                           | GRADE<br>Quality |
|--------------------------------------------------|-------------------------------------------------|------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------|------------------|
|                                                  |                                                 |                                                                                    | management<br>remain standards<br>of care                                                                                                              |                  |
| CDC Yellow<br>Book 2026 TB<br>guidance           | Public health<br>clinical guidance              | Travellers and<br>clinical<br>populations<br>evaluated for TB<br>infection/disease | Recommends<br>IGRA rather than<br>TST for people ≥5<br>years in low-risk<br>populations; lists<br>preferred shorter<br>latent TB infection<br>regimens | Moderate*        |
| WHO diagnostic<br>tools update, 24<br>March 2026 | WHO diagnostic<br>recommendation<br>news/update | People needing<br>fast, accurate TB<br>diagnosis                                   | WHO<br>recommended<br>new diagnostic<br>tools intended to<br>bring rapid<br>accurate<br>diagnosis closer<br>to people                                  | Low–Moderate*    |

\*GRADE quality was assigned at report level when formal GRADE ratings were not visible in the supplied summaries. Explicit high-certainty evidence for RATIONS is from WHO Web Annex D [WHO Web Annex D PDF](#). Additional supporting sources: [WHO news](#), [Mayo Clinic PDF](#), [PLOS Global Public Health](#), [MDPI Tropical Medicine and Infectious Disease](#), [CDC 2025 treatment-guidelines letter](#), [CDC Yellow Book 2026 / NCBI Bookshelf](#), [WHO news](#).

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## 2.3 Evidence Gaps and Future Research Needs

| Domain                        | Evidence gap                                                                                                                                                                                               | Clinical / public health implication                                                                | Suggested priority                                          | GRADE confidence in current evidence                   |
|-------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------|-------------------------------------------------------------|--------------------------------------------------------|
| TB and undernutrition         | Strong RATIONS evidence supports food assistance for contacts in food-insecure settings, but generalizability to other countries, urban settings, and different food-assistance models requires evaluation | Programmes need context-specific implementation models                                              | Pragmatic trials and implementation studies                 | High for RATIONS effect; Moderate for generalizability |
| Micronutrient supplementation | Heterogeneous formulations, doses, schedules, and outcomes; effect estimates not fully visible in supplied summaries                                                                                       | Unclear which micronutrient regimens improve mortality, cure, functional status, or quality of life | Standardized RCTs and individual patient data meta-analysis | Low–Moderate                                           |
| TB and mental health          | WHO recognizes                                                                                                                                                                                             | Mental health screening and                                                                         | Integrated TB–mental                                        | Low                                                    |

| Domain                         | Evidence gap                                                                                                                                                          | Clinical / public health implication                                                | Suggested priority                                                                          | GRADE confidence in current evidence |
|--------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------|--------------------------------------|
|                                | mental health conditions as associated with higher mortality, poorer treatment outcomes, and lower quality of life, but detailed formal GRADE tables were not visible | referral may be under-implemented in TB services                                    | health trials and implementation studies                                                    |                                      |
| TB, alcohol, and substance use | Evidence supports links to worse engagement, immune compromise, malnutrition, and drug–drug interactions, but programme models vary                                   | Substance-use treatment should be integrated with TB care where feasible            | Trials of opioid agonist therapy integration and alcohol-use interventions in TB programmes | Low                                  |
| Diabetes and TB                | Diabetes is a major risk factor but bidirectional screening implementation remains inconsistent                                                                       | Missed diabetes worsens TB outcomes; missed TB in diabetes clinics delays diagnosis | Cost-effectiveness and implementation research for bidirectional screening                  | Moderate                             |

| Domain                           | Evidence gap                                                                                                                  | Clinical / public health implication                                                    | Suggested priority                                                                                        | GRADE confidence in current evidence |
|----------------------------------|-------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------|--------------------------------------|
| Post-TB lung disease             | 2025 scoping review noted absence of WHO PTLD guidelines at the time                                                          | Survivors may have persistent respiratory impairment without standardized care pathways | WHO guideline development and trials of pulmonary rehabilitation, inhaled therapies, and follow-up models | Low                                  |
| Adolescents and youth engagement | WHO has adolescent-friendly TB service content, but no TB-specific WHO youth-engagement case study was identified             | Youth voices may be underrepresented in TB guideline development and implementation     | TB-specific meaningful youth engagement framework and evaluation                                          | Very low                             |
| New diagnostics                  | WHO recommended new diagnostic tools in 2026, but complete recommendation tables and implementation evidence were not visible | Programmes need cost, infrastructure, and equity assessments                            | Diagnostic accuracy, impact, and cost-effectiveness studies                                               | Low–Moderate                         |

## 2.4 Updated Recommendations Based on New Evidence

| Original Recommendation                                            | New Evidence Impact                                                                                                       | Updated Recommendation                                                                                                                                 | GRADE Evidence Quality                                       | Strength                                         |
|--------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------|--------------------------------------------------|
| Address undernutrition as part of people-centred TB care.          | RATIONS showed food/nutritional support reduced incident TB among household contacts, <b>aIRR 0.61, 95% CI 0.43–0.85.</b> | In food-insecure settings, provide food assistance to household contacts of people with TB as part of TB prevention packages.                          | <b>High</b>                                                  | Strong                                           |
| Provide nutritional assessment and counselling for people with TB. | WHO 2025 undernutrition recommendations extend attention to household contacts and prevention.                            | Assess nutritional status and provide counselling for all people with TB and household contacts; link undernourished individuals to nutrition support. | Moderate*                                                    | Strong                                           |
| Manage TB and HIV collaboratively.                                 | Module 6 confirms high-certainty evidence that people living with HIV should receive at least the same                    | Maintain equal-duration daily TB treatment for people living with HIV; integrate TB treatment into HIV care settings where feasible in                 | High for treatment duration; Low for service-delivery model* | Strong for duration; Conditional for integration |

| <b>Original Recommendation</b>                                                        | <b>New Evidence Impact</b>                                                                                                            | <b>Updated Recommendation</b>                                                                                                                                                                  | <b>GRADE Evidence Quality</b> | <b>Strength</b> |
|---------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------|-----------------|
|                                                                                       | duration of daily TB treatment as HIV-negative patients.                                                                              | high-burden settings.                                                                                                                                                                          |                               |                 |
| Use WHO-recommended diagnostics for rapid TB detection and drug-resistance detection. | WHO updated Module 3 in 2025 and announced new diagnostic tools in March 2026.                                                        | Countries should update national diagnostic algorithms to include WHO-recommended rapid diagnostic tools, with attention to access, equity, sample type, and drug-resistance detection.        | Moderate*                     | Strong*         |
| Treat TB using WHO-recommended regimens and patient support.                          | WHO Module 4 consolidated treatment and care in 2025; CDC/ATS/ERS/ IDSA 2025 guidance supports shorter regimens in selected settings. | Use WHO Module 4 regimens as the global standard; consider shorter regimens only for eligible patients and where drug-susceptibility testing, monitoring, and adherence support are available. | Moderate*                     | Strong*         |
| Provide                                                                               | WHO TB                                                                                                                                | TB programmes                                                                                                                                                                                  | Low*                          | Conditional     |

| Original Recommendation          | New Evidence Impact                                                                                                           | Updated Recommendation                                                                                                                                                                    | GRADE Evidence Quality | Strength    |
|----------------------------------|-------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------|-------------|
| adolescent-friendly TB services. | Knowledge Sharing Platform highlights adolescent needs; no TB-specific meaningful youth-engagement case study was identified. | should design adolescent-friendly services and include adolescents/youth in guideline adaptation and service-design processes using WHO's broader meaningful youth-engagement principles. |                        |             |
| Address post-TB lung health.     | 2025 scoping review identified PTLD as a guideline gap.                                                                       | Establish post-treatment assessment pathways for respiratory symptoms and functional impairment while awaiting WHO PTLD-specific guidance.                                                | Low                    | Conditional |

\*Formal WHO GRADE tables were not fully visible in the supplied summaries; these are report-level GRADE judgments.

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## Bottom-Line Clinical Guidance as of 29 April 2026

1. **Diagnose early and rapidly:** implement WHO Module 3 diagnostic recommendations, including rapid tools for TB detection and rifampicin/drug-resistance detection [WHO news](#).
2. **Treat according to WHO Module 4:** use WHO-recommended regimens for drug-susceptible and drug-resistant TB, with integrated patient care and support [WHO Module 4 publication page](#).
3. **Integrate comorbidity care:** screen and manage HIV, diabetes, undernutrition, mental health, tobacco use, alcohol/substance use, and lung health within TB services wherever feasible [WHO operational handbook](#).
4. **Prioritize nutrition:** provide nutritional assessment and counselling for people with TB and contacts; provide food assistance for household contacts in food-insecure settings, supported by high-certainty RATIONS trial evidence [WHO Web Annex D PDF](#).
5. **Maintain strong TB/HIV standards:** people living with HIV and TB should receive at least the same duration of daily TB treatment as HIV-negative TB patients, supported by high-certainty evidence and a strong WHO recommendation [WHO consolidated guidelines PDF](#).
6. **Address adolescents and youth:** implement adolescent-friendly TB services and consider WHO's broader meaningful youth-engagement approach, while recognizing that no TB-specific WHO youth-engagement case study was identified [WHO TB Knowledge Sharing Platform](#), [WHO Western Pacific publication page](#).